Prodigy Anesthesia Quick Review

Splenic Surgery

What are the most common preop diagnoses seen in patients presenting for laparoscopic-assisted splenectomy?	1. Idiopathic thrombocytopenic purpura (ITP), which accounts for 60% of the patients 2.Thrombotic thrombocytopenic purpura (TTP) 3. Lymphoma 4. Hereditary spherocytosis 5. Hypersplenism 6. Hemolytic anemia. *Jaffe RA, Schmiesing CA, Golianu B. Anesthesiologist's Manual of Surgical Procedures. 5th ed. Philadelphia, PA: Wolters Kluwer Health, 2014: 596.*
Currently, what is the only absolute contraindication to the performance of a laparoscopic-assisted splenectomy?	Massive splenomegaly (spleens measuring > 30 cm longitudinally) *Jaffe RA, Schmiesing CA, Golianu B. Anesthesiologist's Manual of Surgical Procedures. 5th ed. Philadelphia, PA: Wolters Kluwer Health, 2014: 595.*
How is a patient placed for a laparoscopic-assisted splenectomy?	On a beanbag in full right lateral decubitus position or on a beanbag in a right 45 degree lateral decubitus position. The kidney rest is elevated in either position, and the table should be flexed. *Jaffe RA, Schmiesing CA, Golianu B. Anesthesiologist's Manual of Surgical Procedures. 5th ed. Philadelphia, PA: Wolters Kluwer Health, 2014: 595-596.*
What is the advantage of placing a patient in a 45 degree right lateral decubitus position as opposed to a full lateral position?	If there is an emergent need for the procedure to be converted to open, the table can easily be rotated and the patient placed in supine position. *Jaffe RA, Schmiesing CA, Golianu B. Anesthesiologist's Manual of Surgical Procedures. 5th ed. Philadelphia, PA: Wolters Kluwer Health, 2014: 595.*
What procedures are performed during a staging laparotomy?	A splenectomy, needle and wedge biopsies of the liver, and biopsies of the celiac, periaortic, portahepatic, and mesenteric lymph nodes. *Jaffe RA, Schmiesing CA, Golianu B. Anesthesiologist's Manual of Surgical Procedures. 5th ed. Philadelphia, PA: Wolters Kluwer Health, 2014: 640.*
What is the most common indication for an open splenectomy?	Traumatic laceration to the spleen *Jaffe RA, Schmiesing CA, Golianu B. Anesthesiologist's Manual of Surgical Procedures. 5th ed. Philadelphia, PA: Wolters Kluwer Health, 2014: 597.*
What type of trauma is most commonly associated with splenic injury?	Blunt trauma *Jaffe RA, Schmiesing CA, Golianu B. Anesthesiologist's Manual of Surgical Procedures. 5th ed. Philadelphia, PA: Wolters Kluwer Health, 2014: 747.*

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List complications of an open splenectomy.	Thrombocytosis, pancreatitis and/or pancreatic fistula, hemorrhage, sepsis, subphrenic abcess, and pulmonary complications (especially in patients being treated with chemotherapeutic agents that cause pulmonary fibrosis) *Jaffe RA, Schmiesing CA, Golianu B. Anesthesiologist's Manual of Surgical Procedures. 5th ed. Philadelphia, PA: Wolters Kluwer Health, 2014: 642, 647.*
What is the primary fluid management concern for a patient undergoing open splenectomy?	The patient is at risk for massive intraoperative blood loss. *Jaffe RA, Schmiesing CA, Golianu B. Anesthesiologist's Manual of Surgical Procedures. 5th ed. Philadelphia, PA: Wolters Kluwer Health, 2014: 646.*

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